



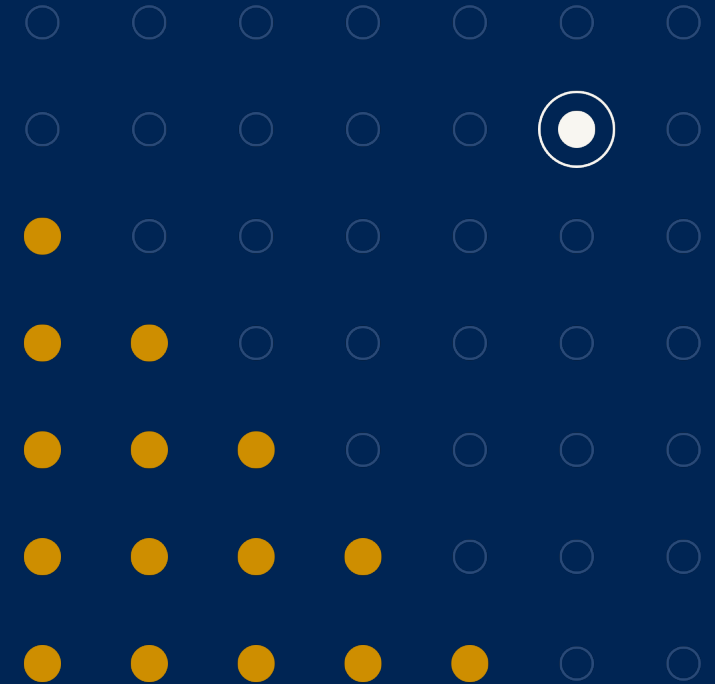
HOME HEALTH · CAPACITY & SCHEDULING INITIATIVE

The people closest to the work, **shaping the choice.**

Introducing the subject matter experts guiding vendor
identification and selection

[Presenter name] · [Date]

Draft for discussion



WHY WE'RE HERE

Give every branch the clarity to grow.

Give PCCs time back for the work only they can do.

Give clinicians an easier start to every day.

This initiative improves how capacity is managed and visits are scheduled, in the branch and in the field, through automation, clear visuals and practical recommendations.

OUR GOALS

What we set out to improve

FOR BRANCHES

See capacity clearly.
Grow with confidence.

- Capacity by discipline and territory, at a glance
- Data to accept referrals with confidence
- Early signals before gaps become missed visits

FOR PCCS

Work at the top of their skills.

- Automation for repetitive, redundant steps
- Visuals and recommendations in one place
- More time for patients, clinicians and care

FOR CLINICIANS

Less time arranging.
More time caring.

- Fewer daily calls to confirm and schedule
- Schedules that respect flexibility
- The reasoning behind every request

OUR PRINCIPLES

What guides every decision

People decide. Tools assist.

Recommendations inform the branch's judgment. They never replace it.

Always show the why.

Every recommendation explains its reasoning, so it can be trusted or challenged.

Protect flexibility.

Autonomy is why clinicians choose home health. We design around it.

Capacity before scheduling.

Get the picture right first, then automate on a foundation people trust.

DUE DILIGENCE

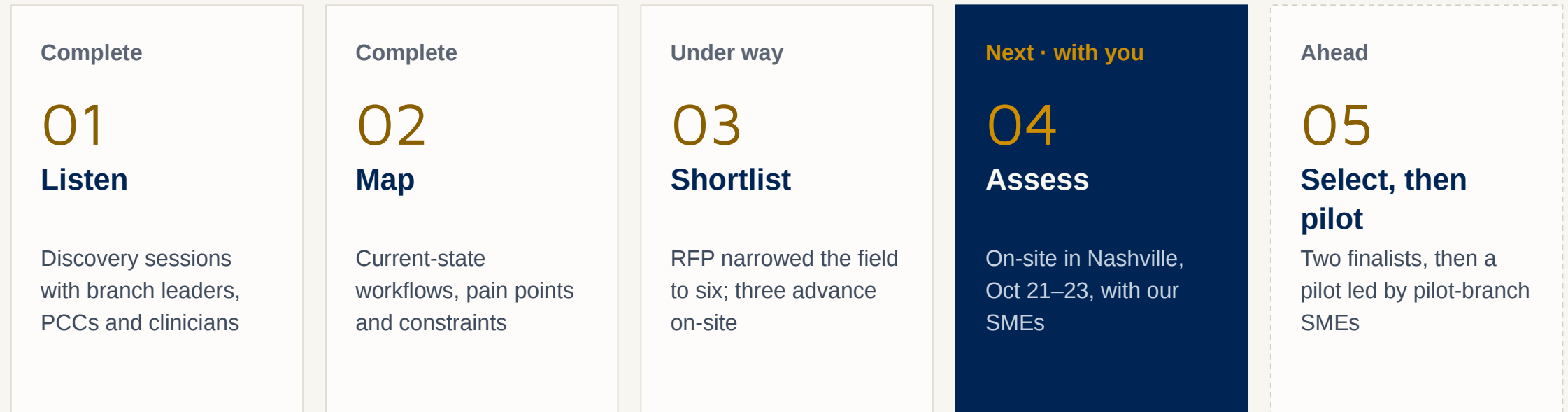
Learning from what came before

Earlier efforts showed us the technology is rarely the hard part. Fit, trust and adoption are.

What we learned	What we are doing differently
Tools were set up to mirror old manual habits	Mapping how our best branches actually work, first
Scheduling was automated before capacity was clear	Capacity visibility comes before automation
Adoption was treated as a rollout step	Adoption is designed in from day one, with the field
Pilots lacked the room and measures to prove out	A defined pilot, clear measures and honest readouts

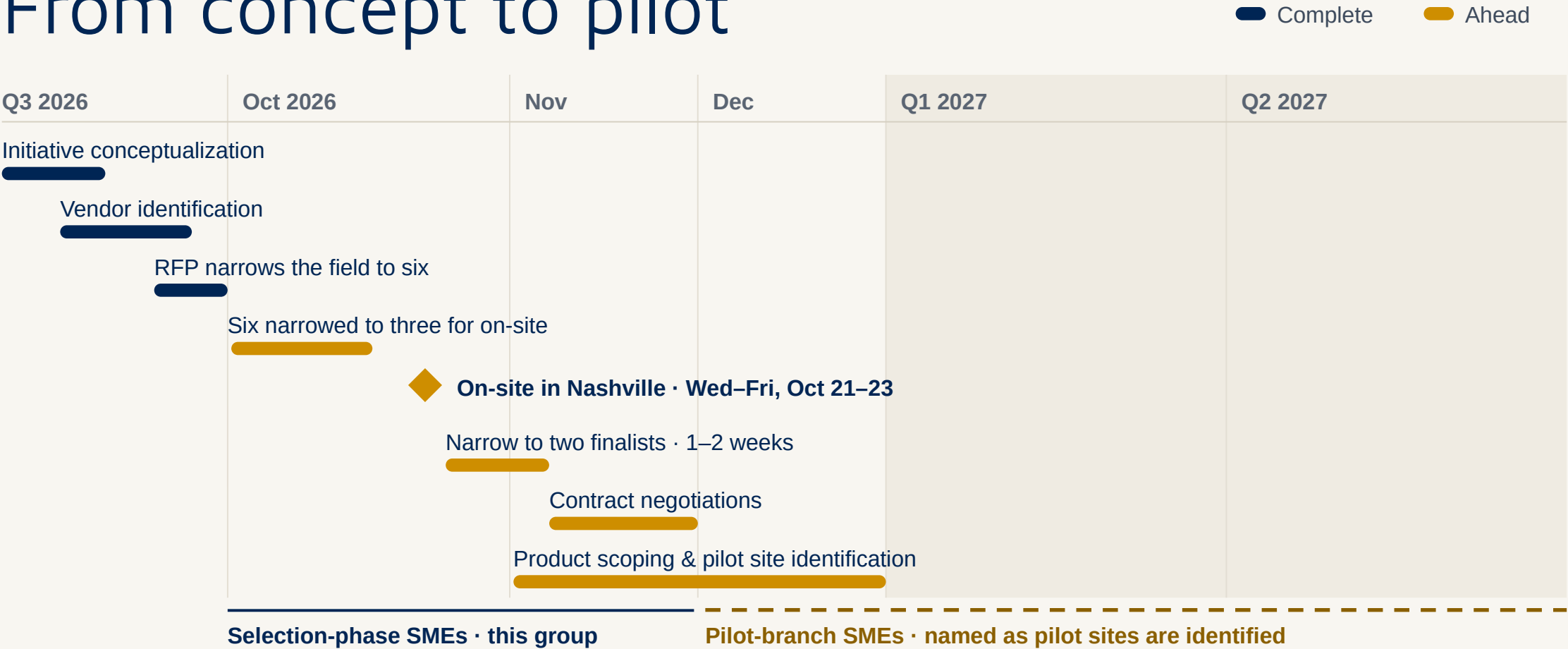
DUE DILIGENCE

Our path to a decision



No decision is made on a demo alone.

From concept to pilot



THE TIMELINE · OPTION B

The road to a pilot



Quarter by quarter

Q3 2026 · DONE • Initiative conceptualization • Vendor identification • RFP narrows the field to six	Q4 2026 · NOW Oct • Six narrowed to three • Nashville on-site · Wed–Fri, Oct 21–23 • Narrow to two · 1–2 weeks Nov • Contract negotiations • Scoping and site identification begin Dec • Product scoping and pilot site identification Selection-phase SMEs · this group	Q1 2027 Pilot-branch SMEs	Q2 2027
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The best judges of a tool are the people who will **use it every day.**

Vendors will show us what their products can do. Our subject matter experts will tell us whether they work here.

Two phases, two groups of experts

NOW · SELECTION PHASE

Vendor selection SMEs

This group. You assess the three finalists on-site in Nashville and guide the choice to two platforms.

- On-site assessments, Oct 21–23
- One shared scorecard and debrief
- Input to the recommendation



NEXT · PILOT PHASE

Pilot-branch SMEs

A second group, drawn from the branches chosen to pilot one of the two selected platforms.

- Named once pilot branches are set
- Shape setup and workflows locally
- Lead adoption with their teams

Who is at the table

Branch Executive Director

Growth, staffing model and the economics of saying yes

Director of Clinical Services

Clinical judgment, start-of-care assignment and compliance

Patient Care Coordinator

The daily scheduling workflow, and exactly where it breaks

Field RN / Start-of-Care Nurse

What earns a clinician's yes, and what a tool must never do

Therapy Lead (PT / OT)

Discipline mix, visit frequency and continuity of care

Workforce & Staffing

Capacity models, flex coverage and per-diem engagement

SELECTION PHASE

Meet our selection-phase experts



[Name]

[Title]

[Branch · Market]

[What they bring]



[Name]

[Title]

[Branch · Market]

[What they bring]



[Name]

[Title]

[Branch · Market]

[What they bring]



[Name]

[Title]

[Branch · Market]

[What they bring]



[Name]

[Title]

[Branch · Market]

[What they bring]



[Name]

[Title]

[Branch · Market]

[What they bring]

What our experts will do

01 Bring real scenarios
Test each product on situations our branches face every week, not a scripted demo.

02 Score it together
Rate every vendor on the same shared scorecard, so comparisons are fair.

03 Find the edges
Push on the exceptions: call-outs, authorizations, recerts and weekend coverage.

04 Speak for the field
Tell us what a tool must always do, and what it must never do.

THE ASSESSMENT LENS

How we will judge fit

What we assess	The question our experts will ask
Real-world fit	Does it work the way our branches and field actually work?
Capacity first	Does it show capacity by discipline and territory before it schedules?
Transparency	Does every recommendation show its reasoning?
Human in control	Can the branch override easily, and does the tool learn from it?
Clinician experience	Does it reduce daily calls and preserve flexibility?
Integration	Does it work with HCHB and the systems we already use?
Ease of adoption	Could a PCC use it with confidence after a short orientation?

THE OUTCOME

What success looks like

FOR A BRANCH LEADER

Capacity by discipline and territory in one view, and the confidence to say yes.

FOR A CLINICIAN

A day that starts confirmed, with fewer calls to make before the first visit.

FOR A PCC

A morning spent coordinating care, not re-entering the same data in several places.

FOR A PATIENT

A visit window they can count on, from a clinician they know.

NEXT STEPS

What happens next

Before Oct 21	SME orientation and scenario prep
Oct 21–23	On-site vendor assessments in Nashville
Late Oct	SME debrief and scorecard review
Early Nov	Two finalists advance to contract negotiations

What we ask of you

- Wed–Fri, Oct 21–23 in Nashville, plus prep and debrief
- Real examples from your branch or caseload
- Candor, especially when something won't work
- Discretion while evaluations are underway

Your experience shapes
this decision.

Thank you for lending it.

[Initiative lead] · [Email]

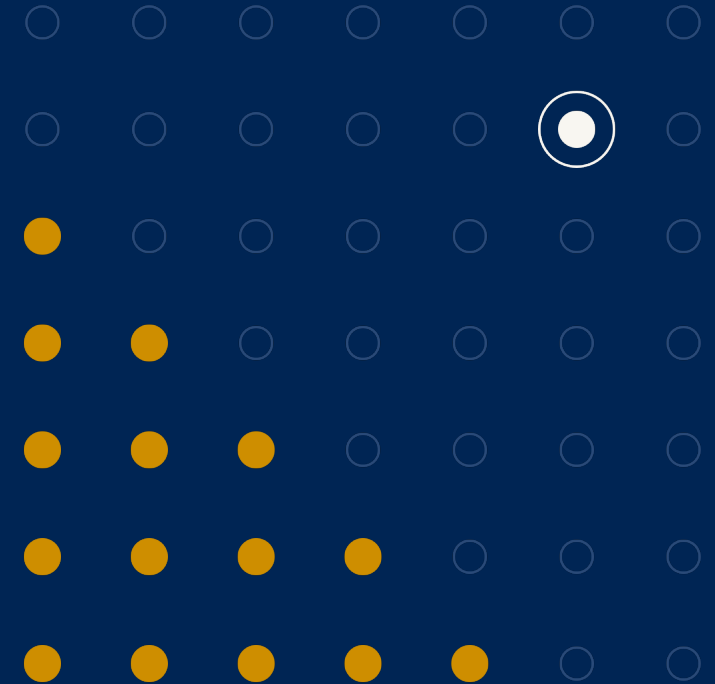




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One line that sets up the section



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Slide title

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